جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نسمه محمد الهادي احمد
المحافظة : القليوبيه **الحي / المدينة :** طوخ **العنوان :** ---
رقم البطاقة : 28808231401442 **رقم التليفون :** 01030199504 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 23-08-1988 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نسمه محمد الهادي احمد
National ID: 28808231401442

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS

← سونار على الشرى والاربط

← سونار بطا وصور



جمعية أنصار السنة المحمدية ببنها

اسم المريض	: نسمه محمد الهادى	الجنسية	: مصر	رقم المريض	: 351075
تاريخ الطلب	: 21/12/2025	تاريخ العينة	: 21/12/2025 14:32	العمر : 36	السنة : 11 الشهر :
الدكتور المنفذ	: سوزان محمد فاروق	تاريخ النتيجة	: 21/12/2025 14:32	الجنس	: أنثى
الدكتور الطالب	: طبيب داخلى			مؤكد النتيجة	: ايمان مصطفى بهاء الدين ابو
اسم الشركة	: نقدى مستشفى ١٥% - ٨٩ نقدى مستشفى ١٥% - ١ نقدى 85			نوع المريض	: نقدى

Calcium

REQ ID 175894

Test	Result	Unit	Reference Range
Calcium Total	10.2	mg/dL	- Newborn 8.5-10.5 Children 10-12 Adult 8.5-10.5

Calcium Ionized

REQ ID 175894

Test	Result	Unit	Reference Range
Calcium Ionized	1.4	mmol/L	1.15-1.32

Blood Urea

REQ ID 175894

Test	Result	Unit	Reference Range
Urea	33	mg/dL	10-50 10-50

Creatinine

REQ ID 175894

Test	Result	Unit	Reference Range
Creatinine	0.9	mg/dL	0.4-1.2

Uric Acid

REQ ID 175894

Test	Result	Unit	Reference Range
Uric Acid	2.1	mg/dL	2.3-6.5

Name: نسمه محمد الهادى	
MR. : 2029	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with a left-sided invasive ductal carcinoma, grade 2, measuring 36 mm, staged as clinical T4c N1. Hormone receptor testing showed estrogen receptor 8/8 and progesterone receptor 8/8, with HER2 negative status. Surgical management included left breast skin-sparing mastectomy (SSM) and sentinel lymph node (SLN) biopsy performed on 3 July 2024. Post-operative adjuvant therapy was initiated with Zoladex (goserelin) and Arimidex (anastrozole), with subsequent addition of Zometa (zoledronic acid) as of September 2025.

The patient continued on Zoladex and Arimidex for a planned three-month course, with prescriptions provided for Arimidex and supportive medications.

The patient was noted to be free today for continuation of Zoladex, Arimidex, and Zometa, along with supportive therapy; weight is 65 kg. The plan is to maintain Arimidex and supportive treatment, with pharmacy coordination for medication dispensing.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

Patient Name	: الأستاذه / نسمه محمد الهادى احمد	Request No.	: 13000265
Patient No.	: 28808231401442	Request Date	: 03/07/2024
Path No.	: 2209/24	Verification Date	: 17/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left simple mastectomy and SLNs.

Gross Examination:
* Procedure: Left simple mastectomy specimen measured 11x9x2 cm with covering skin ellipse measured 11x6 cm.
* Tumor extent: A greyish white tumor bed site.
* Size: 5.5x4x1cm.
* Skin involvement: Nipple and areola were slightly fixed.
* Distance from margins: 0.1 cm away from inked deep margin.
* SLNs: Fibrofatty tissue piece measured 2.5x2x0.5 cm entangling 2 lymph nodes up to 1.5x1x0.3 cm.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
- (actual tumor size 5.5x4 cm), (cellularity 65%).
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
* Angioinvasion: No.
* Ductal carcinoma in situ: Focal (15%).
* Perineural invasion: No.
* Skin involvement: Base of the nipple showed Invasive duct carcinoma GII and focal DCIS. Free epidermis of the nipple. No paget 's disease of the nipple.
* Lymph node involvement: Free SLNs (0/2).
* Resection Margins: Free inked deep margin.
* Histologic response to therapy: Partial pathological response.
* Microcalcifications: No.
* Additional pathologic findings: Fibroadenosis.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / نسمه محمد الهادى احمد	Request No.	: 13000265
Patient No.	: 28808231401442	Request Date	: 03/07/2024
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Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis:
Left breast, simple mastectomy and SLNs Biopsy:
* Invasive duct carcinoma G II with focal ductal carcinoma in situ (15%).
* Base of the nipple showed Invasive duct carcinoma GII and focal DCIS. Free epidermis of the nipple. No paget 's disease of the nipple.
* Free SLNs (0/2).
* Free inked deep margin.
* Partial pathological response to therapy.
* Stage: ypT3, ypN0.
* RCB: 2.435, RCB class II.

Signature
Prof. Dr. Gamal abdelatty.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nesma Mohamed Abdel Hady Ahmed Al Ghannam	Date : 01/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 35 Year(s)
Medical ID: 4660/2023	Pat. ID: 44989

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Known case of left breast cancer on NACTH.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Predominant fibro glandular density (ACR D)
- Still noted contracted left breast diffuse infiltrative malignant process associated with overlying microcalcifications.
- No right suspicious micro calcifications or architectural distortion detected.
- **Complementary ultrasound** :
    - Left breast showing diffuse infiltration by large malignant mass lesion measuring about 5X1.3 cm abutting the underlying muscle.
    - No right cystic or solid mass lesions.
- Normal right skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- No left axillary ,infraclavicular or supraclavicular lymphadneopathy.
- Nonspecific right axillary LN.

Opinion:

- Left breast locally advanced malignant mass lesion with pathological axillary lymph nodes (BIRADS 6)
- Right breast adenosis (BIRADS 2)

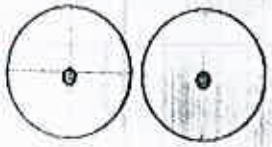
Dr.Yomna Tarek,M.Sc.

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نسمه محمد الهادى احمد Sex: Female
3- Medical number: 83177026
Old Med : 4660/2023
2-ID number: 28808231401442
Patient Age : 36 Y



Family history	+ve	Phone	01030199504	BMI		REASON OF MDT	POST OPERATIVE
marital status	SINGLE	NRS Score		GROUP	II	Date of MDT	03/08/2024
Co morbidity		Menopausal status	PREMENOPAUSAL	Initial STAGE			

Clinical data

Breast Imaging :
LT breast UOQ : solid hyperopic 42*17mm anther small 12*11 lesion
BIRADS

Mets Workup :
PET CT : FREE

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	1		40
BX COMMENT: GII e' focal DCIS	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT:	

T	N	M
4c	1	0

Sonomammography

PATIENT HISTORY
pt rceived NACT AC x4+ 4 x taxotere last 25/5/2024
for left simple mastectomy and slnb
YPT3 YPNO

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOR AI + ZOLADEX
PORT
ZOMETA EVERY 6 MONTH

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Presidential Program for Women's Health
Ministry of Health and Population

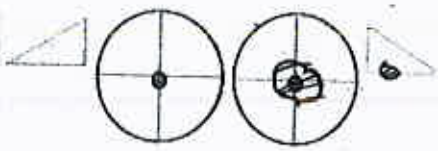
البرنامج الرئاسى لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نسمة محمد الهادى احمد
Sex: Female
3- Medical number: 83177026
Old Med : 4660/2023

2-ID number: 28808231401442
Patient Age : 35 Y

MDT



Family history	Family History +VE FH	Phone	Phone 01030199504	BMI	BMI 3	REASON OF MDT	REASON OF MDT NEW
marital status	Marital Status SINGLE	NRS Score	NRS Score	GROUP	GROUP B	Date of MDT	Date Of MDT 14/12/2023
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE Advanced		

Clinical data

Breast Imaging :
LT breast UOQ : solid hyperopic 42*17mm anther small 12*11 lesion BIRADS

Mets Workup :
PET CT : FREE

BX	ER	PR	Her 2 Neu	SISH	KI67
IDC BX COMMENT: GII e' focal DCIS	8/8 ER COMMENT:	8/8 PR COMMENT:	1 Her 2 Neu COMMENT:	SISH COMMENT :	40%

T	N	M
CT4C	1	0 BY PET

Sonomammography

PATIENT HISTORY
BRCA TESTING
RT SONOMAMMO
FERTILITY COUNSELLING
ECHO, LABS

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOR NACT ACX4 TAXOTERE X4 , ZOLADEX FOR OVARIAN FUNCTION THEN MASTECTOMY

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/	RADIOLOGY	RADIOLOGY د/
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient's Name: Nesma Mohamed Al -Hady Ahmed

Date: 29/11/2023

Prof-Dr: Tamer El-Nahas

WHOLE BODY PET/CT EXAMINATION

Clinical history:-

- A 35-year-old female, a case of recently diagnosed left breast cancer (22/11/2023).

Reason for the study:

- Status evaluation.

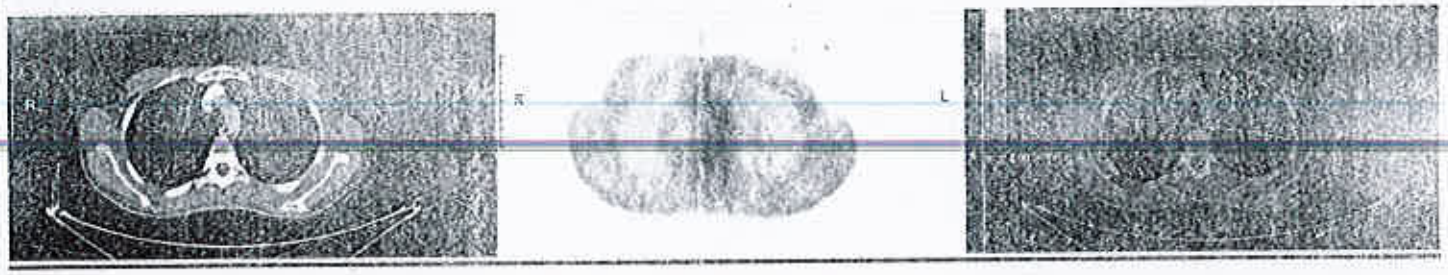
Technique:-

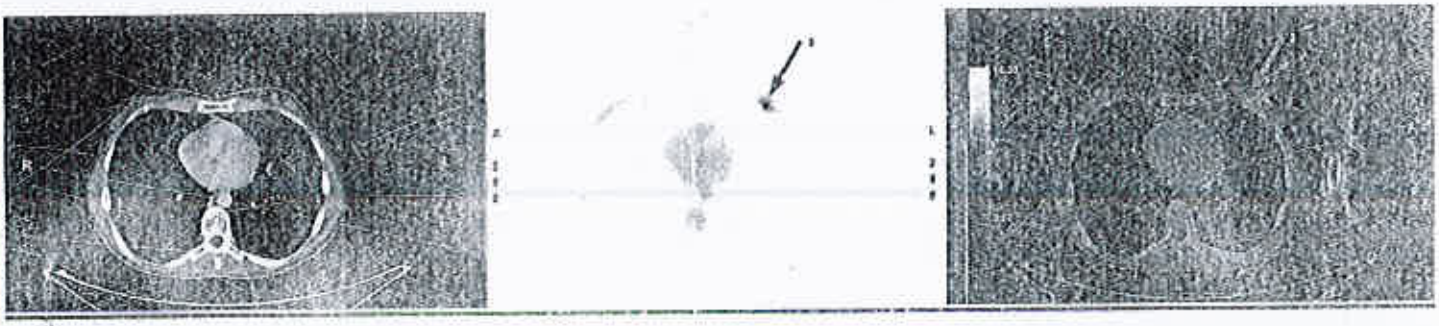
- Whole body PET-CT was acquired with the arms extended above the head after IV injection of the radiopharmaceutical.
- CT was acquired for anatomic localization, attenuation correction, and diagnostic purposes.
- Data were processed and displayed according to departmental protocol.
- Calculation of SUV max. would be done within the region/s of interest whenever needed ($N \leq 3.0$) that is equivalent to the liver SUV max. (internal reference organ).
- **Radiopharmaceutical**: F-18-FDG 8 mCi injected I.V.

FINDINGS:-

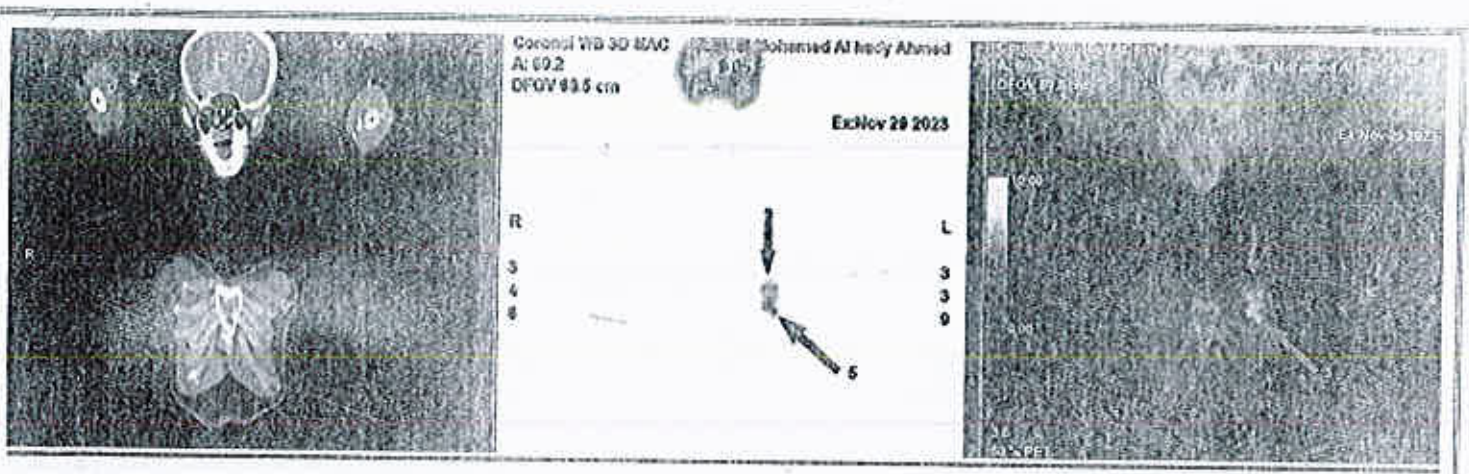
I. Both breasts & regional LNs (primary site of concern):-

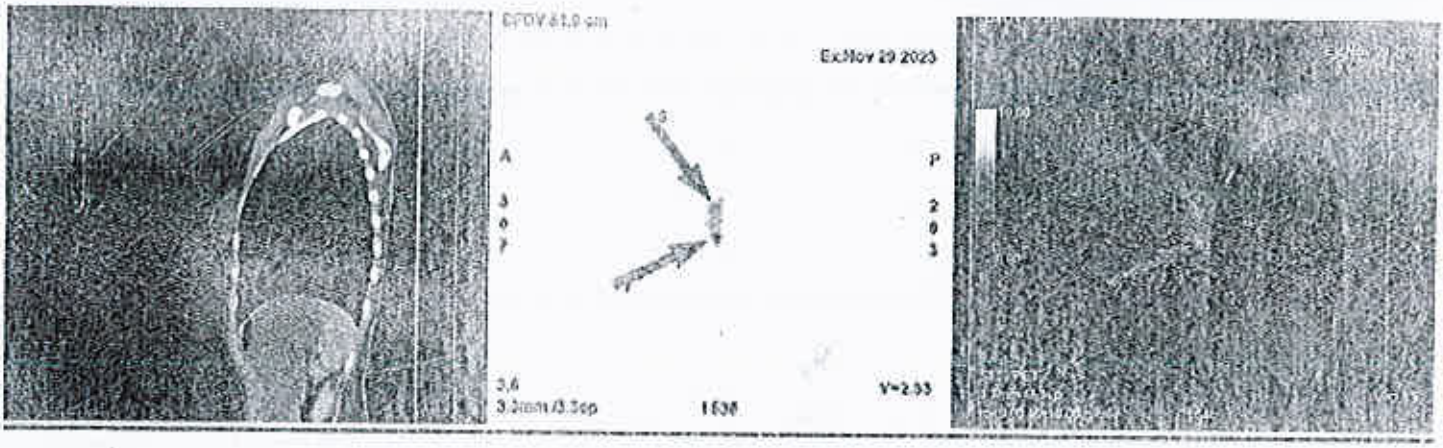
- ***A metabolically active fairly defined speculated margins left breast retro areolar mass lesion is seen, inseparable from the thickened overlying skin, resting on/infiltrating the chest wall muscle, measuring about 44X221 mm, with AUV max of 13.5, a nearby satellite nodule is seen, measuring about 16X13 mm with SUV max of 12.6.***
- ***Small-sized left axillary LNs are seen, largest about 9 mm, with low glucose avidity and SUV max of 3.2.***
- ***No pathologically enlarged or metabolically active right axillary LNs detected.***
- ***No masses or metabolically active lesions could be seen within the right breast.***





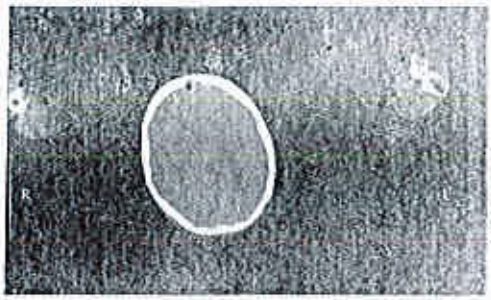


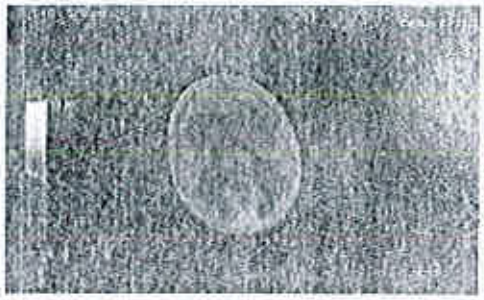




II. Head and neck:-

- ***Diffuse & symmetrical tracer utilization is seen throughout the brain with no metabolically active lesions detected.***
- No pathologically enlarged or metabolically active cervical LNs detected.
- No neck masses or metabolically active lesions were detected.
- Physiologic tracer utilization is seen at the submandibular and salivary glands with no metabolically active lesions detected.
- Normal physiologic tracer utilization is seen at the nasopharynx, oropharynx, and larynx with no metabolically active lesions detected.
- Average-sized thyroid gland with symmetric homogenous tracer utilization all over , no metabolically active lesions detected.



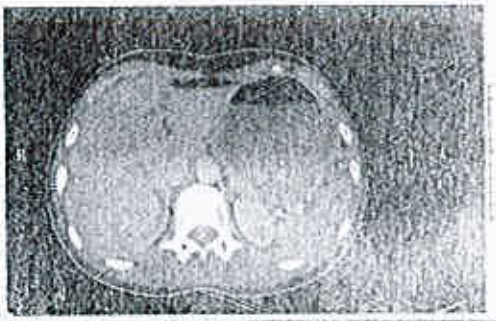


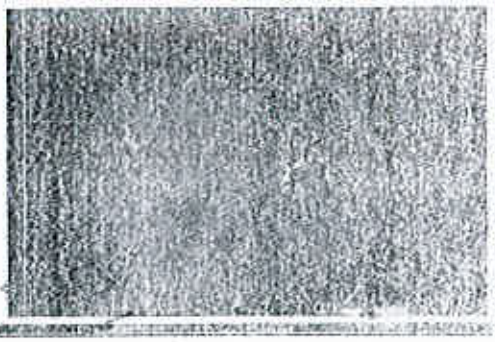
III. Chest:-

- *Clear both lung fields with no metabolically active nodules or masses detected.*
- No pathologically enlarged or metabolically active hilar or mediastinal lymph nodes were seen.
- No metabolically active myocardial or pleural effusion & thickening was seen.
- Physiologic cardiac tracer utilization is noted.

IV. Abdomen and pelvis:-

- *Physiologic diffuse tracer utilization is noted over the average-sized liver showing no metabolically active lesions.*
- *No abdominal or pelvic lymphadenopathy or collections were detected.*
- *No ascites.*
- Physiologic tracer distribution is seen over the average-sized spleen with no definite focal lesions seen.
- Normal appearance of the pancreas with no metabolically active lesions detected.
- Normal size, parenchymal thickness & density of both kidneys.
- Normal appearance of the pelvic organs.





V.Scanned axial and Appendicular skeleton:-

- *No metabolically active osseous lesions could be detected.*

Conclusion:

- ***Metabolically active left breast retro areolar mas lesion associated with a nearby smaller lesion (multifocal disease), associated with overlying skin thickening as well as small left axillary LNs showing mild glucose avidity.***
- ***Rest of the surveyed body shows no scintigraphic evidence of any other significant glucose avid lesions, including the rest of the surveyed LN groups, right breast, brain, lungs, liver, and bones.***
- ***Follow-up PET CT as clinically indicated would be advisable.***

Much Obliged;
Dr. Ashraf Saleh
Nuclear Medicine Consultant

For Further medical information or if you have any questions please do not hesitate to contact me.
Dr. Ahmed Waheed
01005055422


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٢٠١٩/١٠
حاصله على بكالوريوس
سياحة وفنادق
أنثى
مسلمة
أنسة
١١
البطاقة سارية حتى ٢٠٢٦/١٠/١٢


جمهورية مصر العربية
بطاقة تحقيق الشخصية
اسمه
محمد الهادى احمد الظام
عزبة الظام-النحصه
مركز طوخ - القليوبية
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